SUBJECTIVE: The patient is a 47-year-old warehouse supervisor presenting with severe lower back pain and sharp, shooting lancinating pain radiating down his right posterolateral thigh, calf, and into the dorsum of the right foot and great toe for five weeks. He reports the pain began acutely while lifting a heavy crate. The radicular symptoms are aggravated by sitting, coughing, or sneezing, and partially alleviated by lying flat with knees bent. He reports intermittent numbness over the lateral right lower leg. He adamantly denies bowel or bladder incontinence, saddle paresthesias, or lower extremity progressive motor weakness.

PHYSICAL EXAMINATION:

Gait: Antalgic gait favoring the right lower extremity. Able to ambulate on heels, but demonstrates mild weakness on right great toe extension (EHL 4/5 on right vs 5/5 on left).

Spine: Loss of normal lumbar lordosis with marked paravertebral muscle spasms over the right lumbosacral region. Range of motion: Lumbar flexion restricted to 30 degrees secondary to severe radiating calf pain; extension limited to 10 degrees.

Neurological:

- Straight Leg Raise (Lasegue) test is positive on the right at 35 degrees, reproducing classic radicular pain down into the great toe; contralateral (crossed) straight leg raise is positive on the left at 45 degrees reproducing right leg pain.

- Sensory: Decreased pinprick sensation along the right L5 dermatome over the lateral calf and first webspace of the foot.

- Deep Tendon Reflexes: Patellar (L4) 2+ bilaterally, Achilles (S1) 2+ bilaterally and symmetric. Babinski reflex is flexor bilaterally.

MAGNETIC RESONANCE IMAGING (MRI) REVIEW:

Sagittal and axial T2-weighted MRI sequences of the lumbar spine demonstrate preserved disc height at L1-L2, L2-L3, and L3-L4. At the L4-L5 level, there is a prominent, right paracentral extruded disc herniation measuring 7 x 9 mm with inferior migration, causing severe right subarticular recess stenosis and marked mechanical compression of the traversing right L5 nerve root. At L5-S1, there is mild disc desiccation without focal herniation or thecal sac compromise.

PLAN AND RECOMMENDATIONS:

1. Acute right L5 radiculopathy secondary to L4-L5 paracentral extruded intervertebral disc herniation.

2. Given absence of cauda equina syndrome or severe progressive motor deficit, discuss conservative management versus operative intervention.

3. Initiate a short course of oral methylprednisolone dose pack and scheduled gabapentin (300 mg TID).

4. Referral to interventional spine for a right L5 fluoroscopy-guided transforaminal epidural steroid injection (TFESI).

5. If recalcitrant radicular pain persists beyond 6 weeks or neurologic deficits progress, proceed with right-sided L4-L5 minimally invasive lumbar microdiscectomy.